

ANNUAL REPORT 2024



On a Journey of Growth, One Step at a Time

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ACRONYMS

AGYW	Adolescent Girls and Young Women
ANC	Antenatal Care
CSOs	Civil Society Organizations
DICE	Drop-In Centre
FSW	Female Sex Worker
FTYM	First Time Young Mother
HCPs	Health care providers
HIV	Human Immunodeficiency Virus
HSC	Heat Stable Carbetocin
HTS	HIV testing services
ICRHK	International Centre for Reproductive Health-Kenya
IGA	Income-generating activities
KDHS	Kenya Demographic and Health Survey
KNBS	Kenya National Bureau of Statistics
KVP	Key and Vulnerable Populations
MSM	Men who have Sex with Men
NASCOP	National Aids and STI Control Program
PLHIV	People Living With HIV
PMA	Performance Monitoring Action
PMAP	Postpartum Hemorrhage Medicines Accelerate Project
PPH	Postpartum Hemorrhage
PNC	Post-natal care
PrEP	Pre-exposure prophylaxis
PSK	Population Services Kenya
RMNCAH	Reproductive, Maternal, Newborn, Child, and Adolescent Health
SRH	Sexual and Reproductive Health
SRHR	Sexual and Reproductive Health Rights
TXA	Tranexamic acid
UNFPA	United Nations Population Fund



EXECUTIVE SUMMARY

For over 25 years, International Centre for Reproductive Health Kenya (ICRHK) has remained dedicated to advancing sexual and reproductive health and rights (SRHR), particularly for women, adolescents, and marginalized communities.

Our work is built on a strong foundation of research, advocacy, and program implementation, ensuring that the most vulnerable populations have equitable access to quality healthcare.

Despite progress, SRHR challenges persist. Limited access to comprehensive services, high adolescent pregnancy rates, gender-based violence, and barriers to contraception continue to impact vulnerable populations. Additionally, stigma, discrimination, funding constraints, and shifting global health priorities present ongoing hurdles to sustainability.

Message from Our Country Director

As we reflect on a year of remarkable achievements, I am filled with immense pride and a profound sense of hope. We not only met the challenges before us but surpassed expectations, demonstrating an unwavering commitment to creating lasting, positive change.

Our impact is undeniable, extending vital services and support to those who need it most across 33 counties. We provided HIV/AIDS services to 16,945 clients, delivered adolescent sexual and reproductive health (ASRH) services to 4,384 young people—including new mothers, offered a lifeline to 365 individuals through comprehensive gender-based violence (GBV) services and trained 693 health workers on Heat Stable Carbetocin and Tranexamic Acid through the national EmONC mentorship package strengthening integrated postpartum prevention and treatment. Furthermore, our “Linda Mtoto” program empowered over 500 young mothers to build brighter futures by awarding 103 bursaries.

Beyond direct service delivery, our influence extends to shaping national policies. We worked closely with the Ministry of Health to support the development of four new health-related guidelines, cementing our commitment to evidence-based interventions. This is further demonstrated by our crucial studies on menstrual health, disability inclusion in RMNCAH services, and the piloting of STI diagnostics. We also continue to contribute valuable data on GBV among young women through our PMA Agile project.

Advancing reproductive health for all is a core focus of our work. Our disability inclusion studies, for example, explored the extent of inclusivity in RMNCH service provision in Kenya to inform policy and ongoing health system-strengthening efforts.

Our work knows no borders. With FCDO funding this year, our team expanded family planning research to Tanzania, where we conducted a study to understand the drivers of low modern contraceptive uptake.



Dr. Ontiri

On the global stage, we shared our insights and contributed to critical conversations by actively participating in nine international conferences and publishing three manuscripts. Our growing impact is reflected in a 33% growth in our annual budget, from \$3.79 million in 2023 to \$5.05 million in 2024. We successfully implemented seven new projects: the Evaluation of the GAVI HSS Programme, Genesis Study, MOMENTUM Country and Global Leadership CGL, Nuru ya Vijana, Okoa Mama na Mtoto Initiative, PPH Carbetocin, and the FP study in Tanzania.

A significant milestone was securing our first direct grant from the Gates Foundation, a strong testament to our growing credibility and institutional capacity.

These achievements are more than just numbers; they represent transformed lives, strengthened communities, and a brighter future for all. We remain steadfast in our vision to implement high-impact research and interventions in sexual and reproductive health.

About ICRHK

Who We Are

International Centre for Reproductive Health- Kenya (ICRHK) is a leading independent local non-governmental organization (NGO) dedicated to advancing sexual and reproductive health and rights (SRHR) through research, advocacy, and interventions. It was established in 2000 in Kenya and is part of the ICRH global network, which also includes independent country offices in Belgium and Mozambique.

Since then, we have expanded our footprint, to drive evidence-based interventions and currently we have our presence in 33 counties across Kenya, bringing life-changing SRHR research and programs to communities.

What We Do



Our Impact

25+ Years of Advancing Quality Care Outcomes

Shaping Policy

Driving national and global health agendas through research & advocacy.

Delivering game-changing solutions in HIV, RH, maternal health, and GBV.

Innovating Care

Expanding Access

Reaching marginalized populations with essential health care services

Building Enduring Partnerships for Long Term Impact

Ensuring Sustainability

Our multidisciplinary team of 51 full-time staff members and over 100 short-term staff, including researchers, clinicians, public health experts, social scientists and program managers who work collaboratively to design and implement interventions and studies. These efforts target marginalized populations, and health care workers ensuring access to comprehensive SRHR services



Vision

To be a centre of excellence in implementation of research and interventions in public health including sexual reproductive health.



Mission

To contribute to improved health status of communities by conducting high quality research, designing and implementing innovative, evidence-based and cost-effective interventions capable of influencing public policy and practice and capacity building in the field of public health including sexual and reproductive health in Kenya.

Our Board Members



**Prof. Kishor
Mandaliya**
Chairperson



Prof. Walter Jaoko
Secretary



Dr. Esther Getambu
Treasurer



Dr. Jenniffer Othigo
Board Member



Mr. Lou Dierick
Board Member



Dr. James Otero
Board Member



Prof. Violet Naanyu
Board Member

Our Senior Management



Dr. Susan Ontiri
Country Director/CEO



Dr. Dismas Congo
Technical/Research Lead



Ms. Mary Thiongo
Project Director



Irene Kiminza, CPA
*Head of Finance/Administration &
Operations*

CHAPTER 1

1.1 Introduction

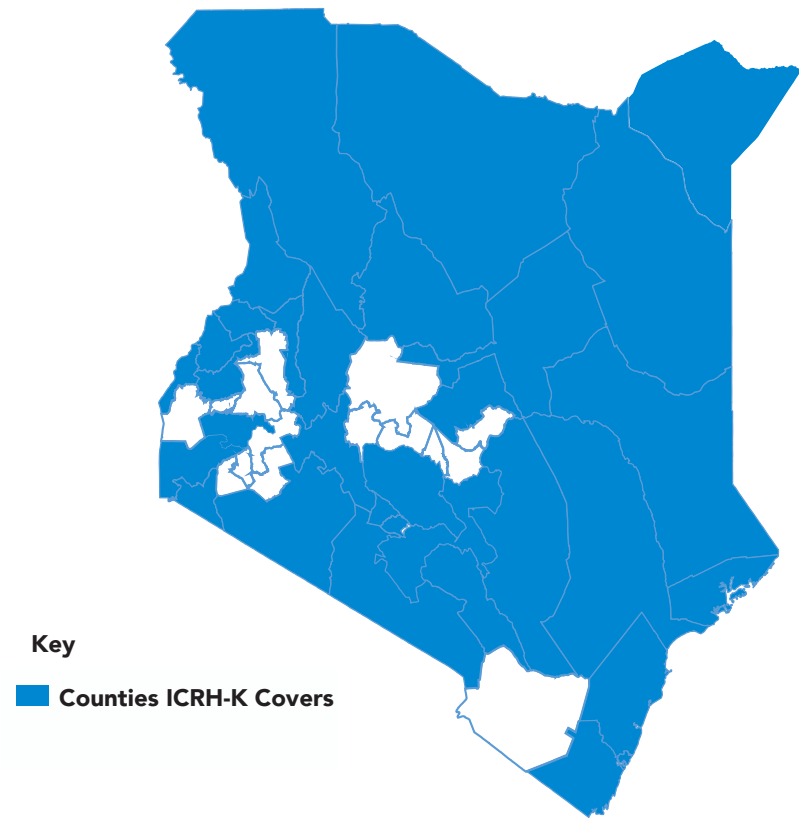
The year 2024 marked a period of significant growth and opportunity for International Centre for Reproductive Health - Kenya (ICRHK).

Our project portfolio expanded substantially, and we secured several new grants, enabling us to broaden our impact across Kenya. ICRHK's projects concentrated on key areas critical to improving public health.

1.2 Our Focus Areas



1.3 ICRHK Coverage in Kenya



1.4 Key Themes

In 2024, ICRHK experienced:

- » Programmatic growth
- » Fostered strategic partnerships
- » Implemented innovative implementation research &
- » Built capacity of health care workers.

CHAPTER

2

At ICRHK, our work has always been driven by a commitment to improving sexual and reproductive health (SRH) outcomes for women, girls, adolescents and youths.

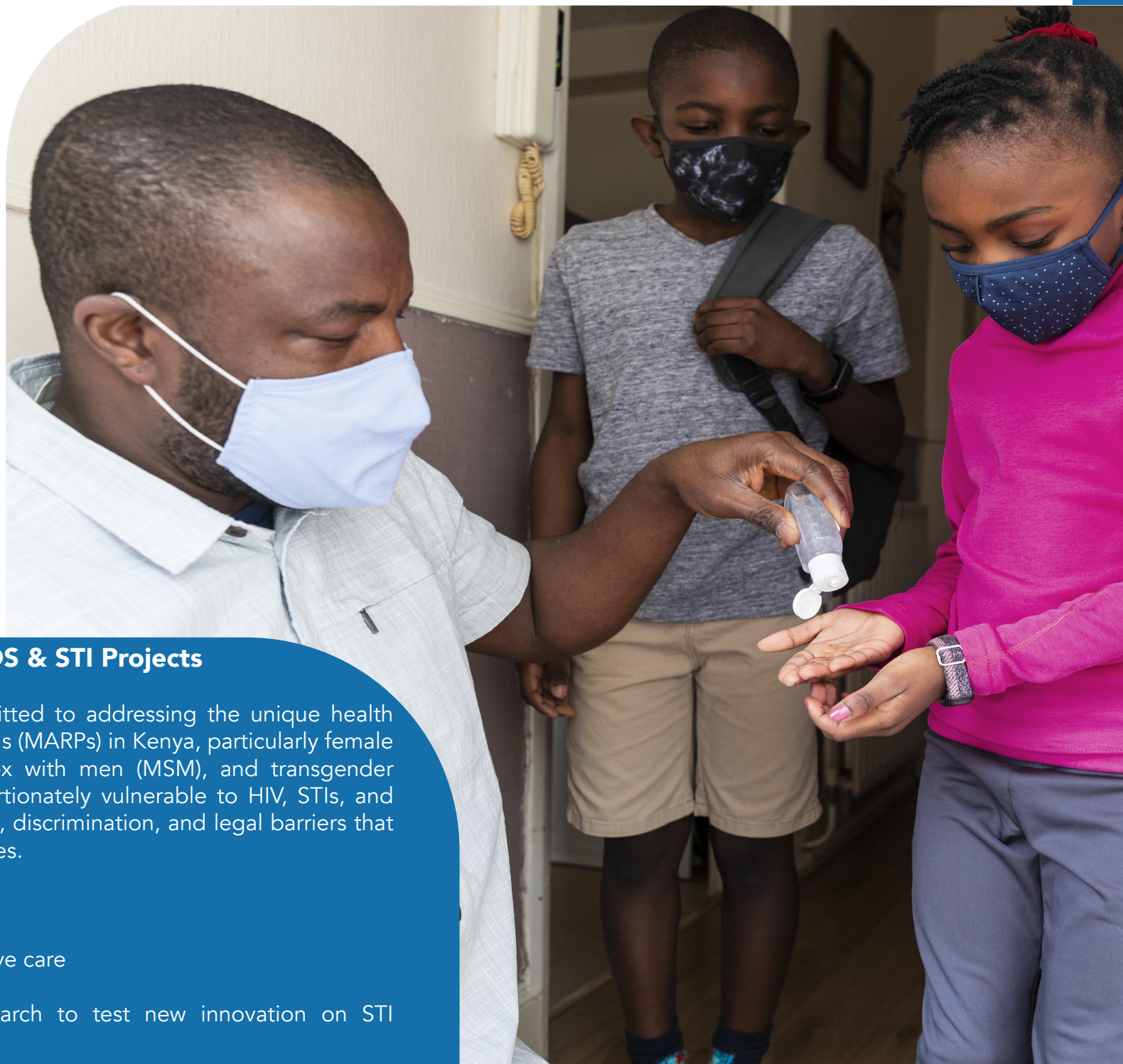
Through innovative programs, extensive research, and strong partnerships, we have made significant strides in advancing Reproductive Health, Maternal Health, and Family Planning, tackling Sexual and Gender-Based Violence (SGBV), providing HIV/AIDS Prevention, Treatment & Care, addressing Adolescent and Youth Health, and Strengthening Health Systems. Below, we share key highlights of our impact over the past year.

2.1 Pillar 1 - HIV/AIDS & STI Projects

Under this thematic pillar, ICRHK is committed to addressing the unique health challenges faced by Most At-Risk Populations (MARPs) in Kenya, particularly female and male sex workers, men who have sex with men (MSM), and transgender individuals. These groups remain disproportionately vulnerable to HIV, STIs, and other health issues due to persistent stigma, discrimination, and legal barriers that hinder their access to essential health services.

Our targeted programs focus on:

- Expanding access to HIV and STI services
- Empowering communities through inclusive care
- Driving policy change through advocacy.
- Implementation of implementation research to test new innovation on STI management and treatment



a. Stawisha Pwani

This is a five-year project initiated in 2021 and has been implemented by ICRHK under the prime awardee LVCT Health. It is funded by the President's Emergency Plan for AIDS Relief (PEPFAR) through the US Agency for International Development (USAID). ICRHK runs six drop-in centers; Mombasa County in Mvita, Likoni, & Kisauni & Kilifi County in Mtwapa, Kilifi and Malindi

Achievements



16,945 key population accessed services

HIV Prevention and Care

14,155 key population tested & received counselling on HIV.

188 newly diagnosed individuals linked to care and treatment.

3,497 initiated on PrEP

2,080 actively on ART at DiCes

1,844 virally suppressed



SRH services

16,945 key population screened for STIs
795 are on treatment

1,468 female sex workers (FSWs) received family planning (FP) services

2,774 survivors accessed post gender-based violence (GBV) care and support.



Contribution to research

5 abstracts on key population health programs were presented.

b. Nuru ya Vijana

Nuru ya Vijana is a 23-month initiative implemented by ICRHK in Kilifi County to improve access to HIV, sexual and reproductive health (SRH), and mental health services for young female sex workers (YFSWs) and young transgender persons (YTGs) aged 15–24. The project is funded by the Federal Ministry of Economic Cooperation and Development, Germany (BMZ), and German Doctors.

Project Scope

- Advocacy for inclusion and protection of youth rights.
- Capacity building for service providers to deliver youth-friendly care.
- Promotion of respectful, person-centered health services.

Milestones



Baseline survey completed

- Insights into the SRH challenges faced by YFSWs and YTGs outlined.



Reach

- 32 youth accessed SRH services, counselling and empowerment.



20 peer educators trained

- Built grassroots network for SRH, HIV and mental health outreach



Community advisory board reconstituted

c. Genesis

The Genesis Project, funded by the Gates Foundation, is a 2.5-year groundbreaking research initiative led by Jhpiego with ICRHK, in collaboration with NASCOP under the Ministry of Health and the County Governments of Nairobi and Kilifi. The project assesses the feasibility, acceptability, and impact of point-of-care (PoC) STI diagnostics to improve timely treatment and strengthen STI management in Kenya's public health system.

Project Scope

- Formative assessment to guide implementation.
- In-depth analysis of integrating PoC STI testing into Kenya's health system.
- Rigorous evaluation of the Aptitude Metrix™ PoC test for accuracy and reliability.
- Field implementation to assess effectiveness, usability, and clinical impact.

Milestones



Formative research initiated

- Ethics approvals secured
- Research team trained



Strategic stakeholder engagement

- Partnerships formed with NASCOP, KLMTB, KEMRI & PPB
- Project aligned with Nairobi & Kilifi County health priorities



Validation & Evaluation prepared

- Protocols developed for lab validation & field studies
- Ethical approvals submitted for both studies



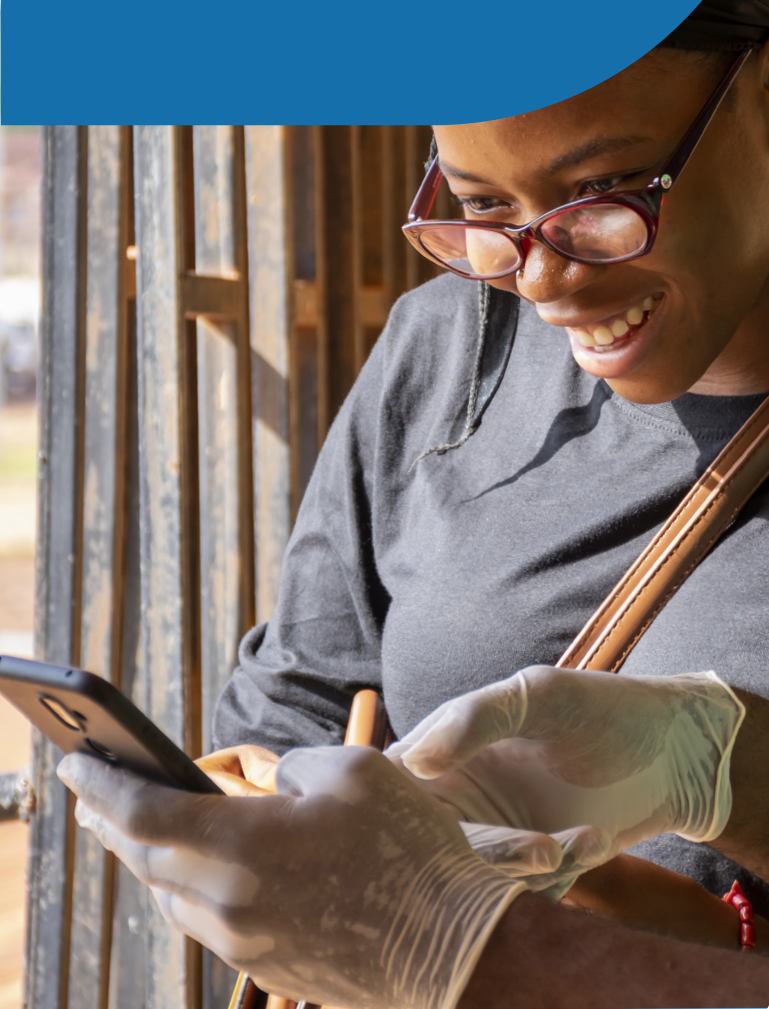
Research Contribution

- 3 scientific abstracts developed



2.2 Pillar 2 - Gender & Gender Based Violence (GBV)

Under Pillar 2: Gender and GBV, we implement projects that generate critical insights into sexual and reproductive health (SRH) and gender-related challenges while strengthening support systems for survivors of violence. These efforts contribute to policy formulation, service improvement, and the empowerment of vulnerable populations, particularly youth and survivors of GBV.



a. PMA Agile Phase 2 Gender and GBV Study

The PMA Agile Phase 2 Gender and GBV Study, funded by the Gates Foundation, builds on research initiated in 2019 to generate evidence that informs policies and programs promoting gender equality and reproductive health in Kenya. This second phase focused on advancing understanding of gender norms and Gender-Based Violence (GBV) by examining new trends, local contexts, and improved measurement approaches.

Over the past year, the study deepened evidence on GBV through both secondary data analysis and new research conducted in Bungoma, Kakamega, Kiambu, and West Pokot. Key areas of focus included intimate partner violence, reproductive coercion, economic abuse, technology-enabled violence, help-seeking, and access to justice. By combining rigorous analysis with fresh data collection, the study provided timely insights to strengthen GBV prevention and response.

A central achievement was ensuring that evidence translated into practice. Findings were strategically disseminated in partnership with youth leaders, gender advocacy networks, policymakers, and grassroots actors, making the data not only accessible but actionable. This intentional approach helped bridge the gap between research and implementation.

In addition, the study invested in building national leadership in evidence-based GBV response. Through its scholar program, it supported a new generation of Kenyan researchers committed to driving sustainable change from within. By pairing strong research with local leadership, PMA Agile Phase 2 continues to position Kenya at the forefront of innovative and impactful GBV response strategies.

Achievements



Research & data use

- 2 major surveys completed



Policy & advocacy

- Informed Kenya's 2024 SDG voluntary national review
- Engaged parliament to advocate for gender responsive, evidence-based policies



Contribution to research

- 7 abstracts presented
- 5 peer reviewed journal articles published



b) Gender Based Recovery Centre (GBVRC)

The Gender-Based Violence Recovery Centre (GBVRC) at Coast General Teaching and Referral Hospital (CGTRH) in Mombasa was established in 2007 through a partnership between the Ministry of Health (MoH), ICRHK, and support from DANIDA. Since then, the centre has become a critical model for survivor-centered GBV care in Kenya.

Through ICRHK's support from core funds, GBVRC provides comprehensive, lifesaving services for survivors, including post-rape medical treatment, psychosocial support, and access to justice through trained paralegals. Beyond direct care, the centre advocates for stronger health system responses to GBV and builds the capacity of healthcare providers to deliver dignified, survivor-centered services.

Achievements



10,307 survivors supported

- Accessed comprehensive post sexual violence services



Model centre for Kenya

- Recognized as a national benchmark site for GBV response and training



Stronger legal pathways

- Paralegal integration has strengthened survivors access to justice



Improved case outcomes

- 50% of reported cases advanced to court with 20% reaching verdict

2.3 Pillar 3 - Adolescent and Youth Health

Our Adolescent and Youth Health programs are designed to empower young people by providing access to comprehensive, age-appropriate health services, education, and support.

With a focus on reproductive health, mental health, sexual health, and overall well-being, these programs aim to address the unique needs and challenges faced by adolescents and young people in Kenya.

Our approach is holistic, understanding the interplay between social, cultural, and economic factors that affect the health outcomes of young people.

a) Linda Mtoto

ICRHK with funding from Kindernothilfe (KNH) and in partnership with County Government of Mombasa is spearheading a transformative initiative focused on addressing Comprehensive Sexuality Education (CSE) for children aged 10-17 years in Mombasa County (Kisauni and Jomvu Sub-Counties).

The project seeks to strengthen community structures and ensure the identification, linkage, and support of children at risk of, or affected by, Child Sexual Exploitation and Abuse (CSEC).

Achievements



- **885** CSEC survivors identified and enrolled in the program.
- **81%** accessed counseling services (emotional and psychological support).



- **103** children received educational and TVET scholarships.



- **400** caregivers trained in income-generating activities (IGAs).



- **9** Self-Help Groups (SHGs) registered with Department of Gender and Social Development & received government revolving funds.

Contribution to research



- **2** abstracts on CSEC data developed and presented.
- Peer-reviewed manuscript on CSEC and child protection published.

b) ASRHR

This evaluation assesses Adolescent Sexual and Reproductive Health (ASRH) implemented in four counties: Bungoma, Busia, Homa Bay, and Marsabit. The program targets adolescent girls and young women aged 10-24 years, with the following objectives:

- Measuring the effectiveness of an integrated multi-sectoral public-private partnership model
- Assessing improved access to sexual and reproductive health services, including contraceptives
- Tracking progress in reducing maternal mortality and morbidity

Achievements

- Phase 1 findings shared through Sense-making workshop.
- 4 dissemination sessions conducted.

Contribution to research

- 4 abstracts submitted



c) MOMENTUM Country and Global Leadership (MCGL) Project

ICRHK's MCGL program, funded by USAID through Jhpiego, advanced adolescent sexual and reproductive health by empowering 9 grassroot-led civil society and community organizations to drive evidence-based advocacy at the county level. The initiative focused on building local capacity, fostering cross-sector collaboration, and amplifying citizen voices to influence policies and funding for family planning, MNCH, and immunization.

- Also, under this project we implemented 3 studies that have contributed to the general outcome of the success of the project in 2024.
- Menstrual Health and Hygiene Management (MHHM) Study
- Disability Inclusion Study
- ASRH Study-Understanding positive outliers as part of exemplary counties

Achievements

• Advocacy Impact:

Held 3 county-level meetings and Three Day Advocacy Approach introduced to guide strategic ASRH action.

- **150** CBOs mapped, **9** CSOs & **3** influencers trained on evidence-based advocacy.

• Evidence Generation:

Completed major studies on ASRH and MHHM (1,392 respondents) and mapped 120 ASRH studies across 26 counties.

• Contribution to knowledge:

2 abstracts presented nationally.

- Advocacy manual and policy brief developed

- **6** media shows, **1** podcast, and **2** TikTok campaigns delivered to drive public awareness.

2.4 Pillar 04 - Reproductive, Maternal, Newborn, Child, and Adolescent Health (RMNCAH)

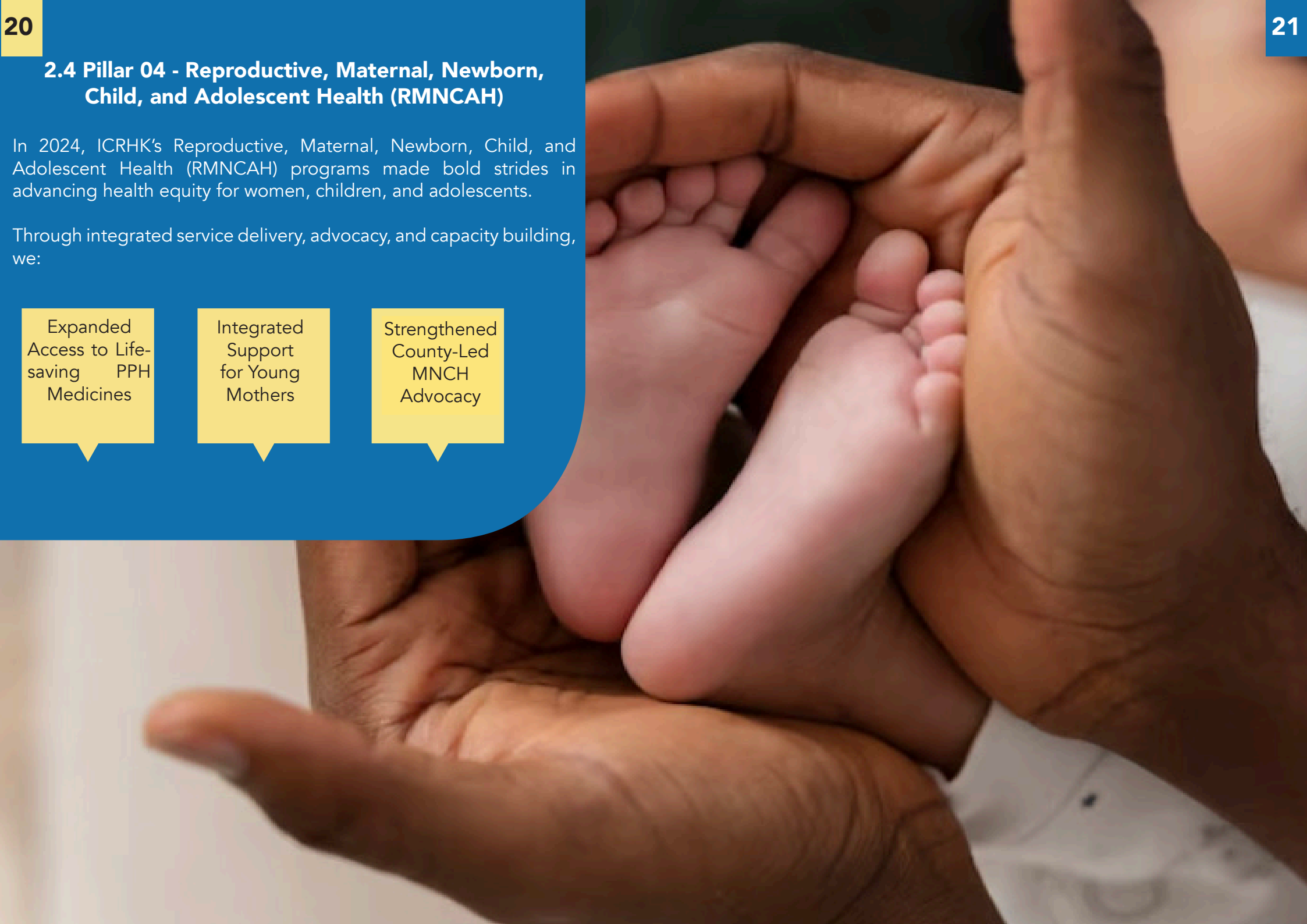
In 2024, ICRHK's Reproductive, Maternal, Newborn, Child, and Adolescent Health (RMNCAH) programs made bold strides in advancing health equity for women, children, and adolescents.

Through integrated service delivery, advocacy, and capacity building, we:

Expanded
Access to Life-
saving PPH
Medicines

Integrated
Support
for Young
Mothers

Strengthened
County-Led
MNCH
Advocacy



a) UNFPA 10th Country program

Under the UNFPA 10th Country Program, ICRHK is driving transformative change in Kenya's Sexual and Reproductive Health and Rights (SRHR) landscape. Our work contributes directly to three national and global priorities:

- Reducing unmet need for family planning
- Preventing maternal deaths
- Eliminating gender-based violence and harmful practices
- ICRHK contributes to the transformative actions by implementing programs under RMNCAH pillar on:



A. First Time Young Mother's Clinic (FTYM)

In partnership with UNFPA and the Kilifi County Department of Health, ICRHK launched the First-Time Young Mothers (FTYM) Clinic to respond to high teen-age pregnancy rates in Kilifi County.

Based at Mtwapa Level 4 Hospital, the initiative delivers a continuum of care for adolescent and young mothers (ages 24 and below), combining:

- Integrated clinical services – antenatal, postnatal, and postpartum family planning.
- Community engagement – raising awareness and linking girls to services.
- Socioeconomic empowerment – equipping young mothers with skills for self-reliance.

Achievements



Health

- **1,300** FTYMs supported
- **100%** hospital deliveries
- **90%** uptake of long-term family planning
- Only **1** HIV -exposed infant positive

Other Achievements



Economic empowerment

- 45 mothers trained in income activities (IGAs)
- 20 IGA startup grants awarded
- 20 enrolled in vocational training (TVET)



Education

- 40 young mothers reintegrated into school



Mental health

- 60 mothers received counselling & support

B. Post-partum Hemorrhage Medicines Accelerate (PMAP)

ICRHK is a crucial partner in Kenya's ongoing efforts to reduce maternal morbidity and mortality. We achieve this through various interventions, including strengthening health systems, advocating for policy changes, and scaling up the use of new and underutilized reproductive health commodities and products.

As part of these initiatives, and with catalytic funding from the Reproductive Health Supplies Coalition, we successfully implemented a project focused on increasing access to new and lesser-used medicines for postpartum hemorrhage (NLUM-PPH). Through this grant, we scaled up the use of Heat-Stable Carbetocin (HSC) for PPH prevention and Tranexamic Acid (TXA) for its treatment.

We integrated these vital medicines as part of our Emergency Obstetric and Newborn Care (EmONC) mentorship programs, ensuring that healthcare providers were trained to use these life-saving products effectively.

Milestones

- Advocacy and data-driven insights led to procurement of an additional 180,000 doses of HSC and an increase in county-level orders, significantly expanding access to this vital medicine.
- Assessed facility readiness for uterotonic management to support the introduction of HSC in 7 implementing counties.
- Trained 693 health workers on HSC and TXA through the national EmONC mentorship package strengthening integrated PPH prevention and treatment across 123 health care facilities from 7 counties
- Facility readiness assessments conducted in 5 counties
- 1500 Health workers in 5 counties mentored on (Helping Mothers Survive –Bleeding After Birth (HMS-BAB) curriculum)

Other Achievements

- Finalized revised National FP Guidelines integrating HRBA, self-care and DM-PA-SC
- Trained 20 national TOTs on MOH Standards (2022) to improve facility-based care.
- Validated and disseminated DMPA -SC costing implementation plan (2024-2030).
- Integrated FP into pre-service curricula in KMTC and selected universities.



C. OMMI Project

Okoa Mama na Mtoto Initiative (OMMI) is a three-year project funded by the Gates foundation. It began in November 2024 and aims to improve maternal and child survival in Kenya through reduction of maternal and child mortalities.

Under OMMI, ICRHK is actively contributing to three strategic MNCH goals targeting 16 priority counties in Kenya:

- Increasing the quantity and quality of domestic funding for MNCH
- Promoting the adoption of scalable policies for women's and child health
- Supporting effective execution of these policies in priority counties

Our Partners

- Ushiriki Wema
- Options Consultancy
- Kenya Obstetrical Gynaecological Society (KOGS)
- KMTC
- Lwala Community Alliance
- Shina Foundation
- Fountain of Hope
- FANIKISHA
- Touch a Life
- Women engaged in development
- Wanahabari



2.5 Pillar 5 - Health Systems Strengthening

ICRHK's commitment to Health Systems Strengthening (HSS) is evident through our Third-Party Monitoring and Evaluation (TPME) work. We independently assess the performance of health programs, ensuring that key components, such as service delivery, financing, and governance, are effectively implemented.



a. Third-Party Monitoring & Evaluation - DESIP

Since 2020, ICRHK has been part of a herera consortium contracted by FCDO/UKAID to conduct Third-Party Monitoring (TPM) and Process Evaluation for the Delivering Sustainable and Equitable Increases in Family Planning (DESIP) project across 19 counties in Kenya.

- As the in-country monitoring and evaluation partner, our role focused on:
- Quarterly TPM & Data Quality Checks – Independent verification of DESIP outputs and data integrity.
- Annual Evaluations – Measured relevance, effectiveness, and sustainability of project implementation.
- Knowledge Sharing – Produced case studies, technical briefs, and hosted webinars for policy learning.

5 years of DESIP implementation

- Key insights generated to inform FCDO and DESIP intervention
- Knowledge products developed: Produced 28 reports, 3 case studies, 2 technical briefs, and co-hosted 5 webinars, 3 workshops, 3 conferences for policy and program learning.
- Field research conducted in 4 counties.

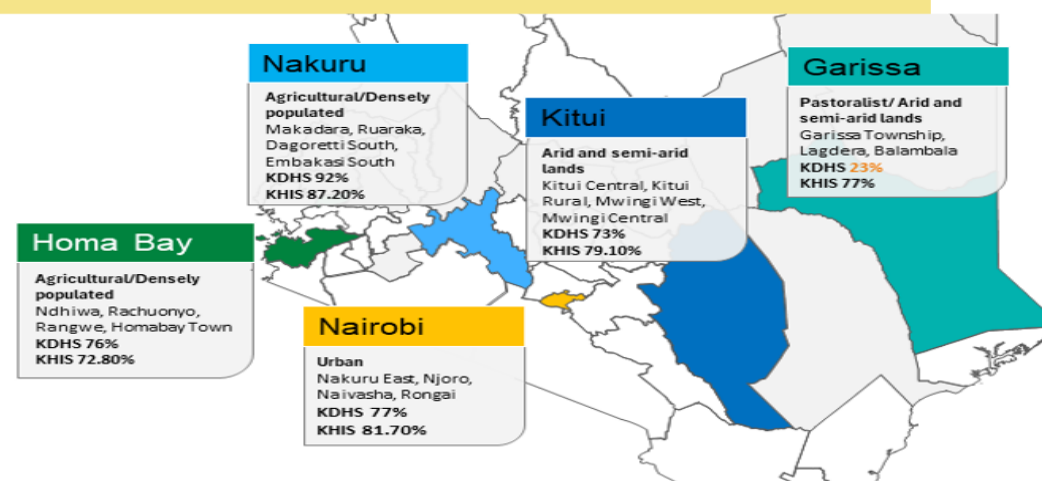
b. Evaluation of Gavi Health Systems Strengthening Support (HSS) in Kenya (2017-2023)

ICRHK played a crucial role in strengthening Kenya's national immunization program by serving as the in-country evaluation partner for the GAVI Health Systems Strengthening (HSS) Grant.

Working with a consortium led by her, ICRHK conducted a comprehensive mixed-method summative evaluation of the grant, which was implemented from November 2017 to September 2023 and managed by UNICEF.

The grant's primary goal was to reduce childhood illness and death from vaccine-preventable diseases by improving the country's immunization system. A key focus was to enhance equitable access and quality of immunization services, particularly for remote and underserved communities, aligning with Kenya's Universal Health Coverage (UHC) strategy.

The evaluation, which was conducted at the national, county, and community levels, assessed the program's relevance, coherence, effectiveness, efficiency, and sustainability, with these five criteria from the OECD/DAC evaluation framework guiding the entire process.



Lessons learned

Community role

Community health promoters played a key role in tracing defaulters, conducting outreaches and strengthening linkages between communities and the health system

Key enablers

Political engagement and use of technology emerged as key enablers for immunization uptake and system accountability

Alignment & coherence

The intervention successfully aligned with national priorities and demonstrated strong coherence with other donor supported health initiatives

Key Contributions & Results

- **Equity in Access:** Enabled routine immunization in 17 counties, reaching mobile, urban poor, and underserved populations.
- **Systems Strengthening:** Improved immunization supply chains, data systems, and service delivery at all levels.
- **Evidence-Based Planning:** Contributed to national evaluation of Gavi HSS impact, efficiency, and relevance in a devolved context.
- **Sustainability & Governance:** Informed assessments on government readiness, domestic financing, and long-term political commitment.

Contribution to Research

Manuscripts published

- <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC11253356/>
- <https://www.sciencedirect.com/science/article/abs/pii/S0145213424000735>
- <https://www.frontiersin.org/journals/psychiatry/articles/10.3389/fpsyt.2023.1209836/full>
- <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4188821/>

Abstracts (Paper titles)

- Implementing a national unique patient identification system among HIV-positive female sex workers in Kilifi, Kenya
- Sex workers' leadership in addressing PrEP continuation barriers in Kilifi County – A case study of ICRH-Kenya's community group
- Effects and perceptions of a peer-led life skills program on prevention and response to commercial sexual exploitation among learners in Mombasa, Kenya
- Undetectable=Untransmittable (U=U) messaging effect on viral suppression, stigma reduction, and community engagement among HIV-positive female sex workers in Mombasa, Kenya
- Facilitators and Barriers of Adolescent Sexual Reproductive Health in Kenya: An Explanatory Sequential Mixed Methods Approach



2024 Snapshot



CHAPTER 4

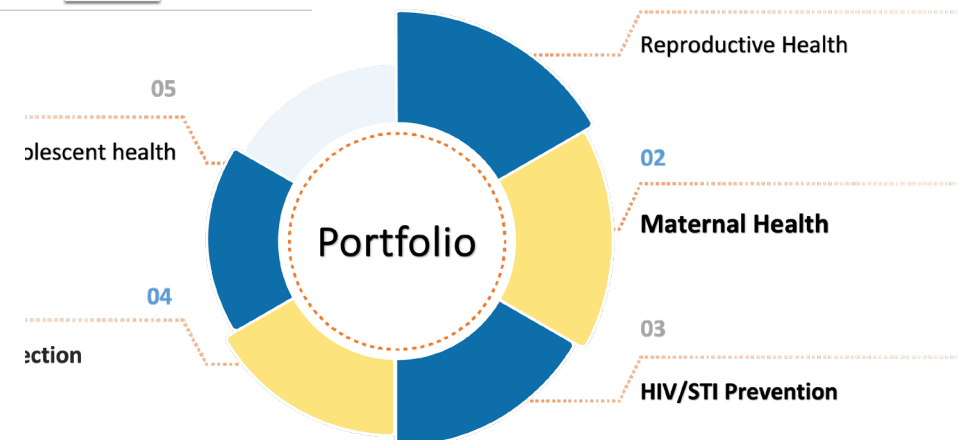
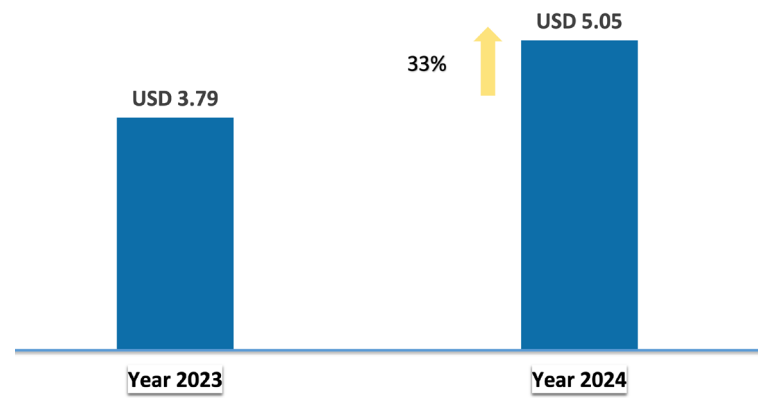


Our Financials

In 2024, ICRHK remained committed to sound financial stewardship, ensuring that the resources entrusted to us were used efficiently to deliver life-changing programs. From the preparation of rigorous budgets to regular monitoring and analysis of the financial results. This ensures that the financial resources are used for the intended purpose and contribute to achieving the organization mission and vision.

The 2024 financial year recorded substantial growth, with both expenses and revenues from donor-funded projects exceeding those of the prior year. This increase was primarily driven by the acquisition of new projects during the period. Overall, revenue grew by 33%, rising from USD 3.79 million in 2023 to USD 5.05 million in 2024. These funds supported a diverse portfolio of initiatives focused on reproductive, maternal health, HIV prevention, adolescent health, and child protection.

Revenue Growth



Our Donors and Partners



USAID
FROM THE AMERICAN PEOPLE



Foreign, Commonwealth
& Development Office



Reproductive Health
SUPPLIES COALITION



equality effect



Federal Ministry
for Economic Cooperation
and Development

BMZ



kinder
not
hilfe



THE AGA KHAN UNIVERSITY



right to health & development



Coast General
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